NCCP 00742 · VERSION 3

Your treatment, explained

NCCP 00742: recurrent malignant glioblastoma; bevacizumab use is off-label. · NCCP 00742 v3 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Lomustine + bevacizumab 5 mg/kg

42-day cycle | Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Call urgently for bleeding, severe abdominal pain, chest symptoms, new headache with visual changes or wound problems. Tell the team about planned surgery before treatment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · lomustine-smpc · avastin · nci-cns

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YOUR COMPLETE REFERENCE

Read what you need, when you need it.

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UNDERSTAND YOUR TREATMENT

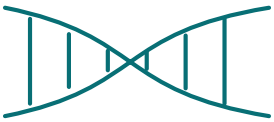
How your medicines work

Lomustine chemotherapy and bevacizumab targeted therapy are used together to help control recurrent glioblastoma. Discuss the expected benefit, symptom goals and treatment burden with your team.

ORAL CHEMOTHERAPY CAPSULE

Lomustine

CCNU



Damages DNA

Lomustine is an oral nitrosourea chemotherapy that can reach the brain. It damages DNA and interferes with cell division. Its effects on healthy blood-forming cells can appear several weeks after the dose and can build up over successive cycles. This explains the long gap between doses and the need for continuing blood tests.

One scheduled dose → DNA damaged → Delayed blood-count effects possible

What to watch for

This is one dose on Day 1, not a daily medicine. If an extra dose is taken, get urgent advice even if you feel well. Keep blood tests after the dose and report fever, bruising, bleeding, new cough or breathlessness.

Sources: nccp · lomustine-smpc

TARGETED ANTIBODY GIVEN INTO A VEIN

Bevacizumab

Avastin / prescribed biosimilar



Blocks the blood-vessel growth signal VEGF

Bevacizumab binds VEGF, a signal involved in making and maintaining blood vessels. Blocking this signal can change the blood supply and leakiness of tumour vessels. It can also affect normal vessels, which helps explain high blood pressure, protein in the urine, bleeding, clots and slower wound healing. A scan change does not by itself prove that the tumour has stopped growing.

VEGF signal blocked → Tumour vessels affected → Response and safety reviewed

What to watch for

Report bleeding, severe abdominal pain, new chest or breathing symptoms, sudden neurological changes, headaches with visual changes, and any operation or wound problem. Keep blood-pressure and urine checks.

Sources: nccp · avastin

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Brain / blood pressure

Bevacizumab / tumour

Keep BP checks. Severe headache, visual change, seizure, sudden weakness or speech difficulty needs urgent assessment.

3 Chest / breathing

Bevacizumab / lomustine

New breathlessness, cough or chest pain needs prompt advice. Severe breathing difficulty needs emergency help.

5 Kidneys / urine

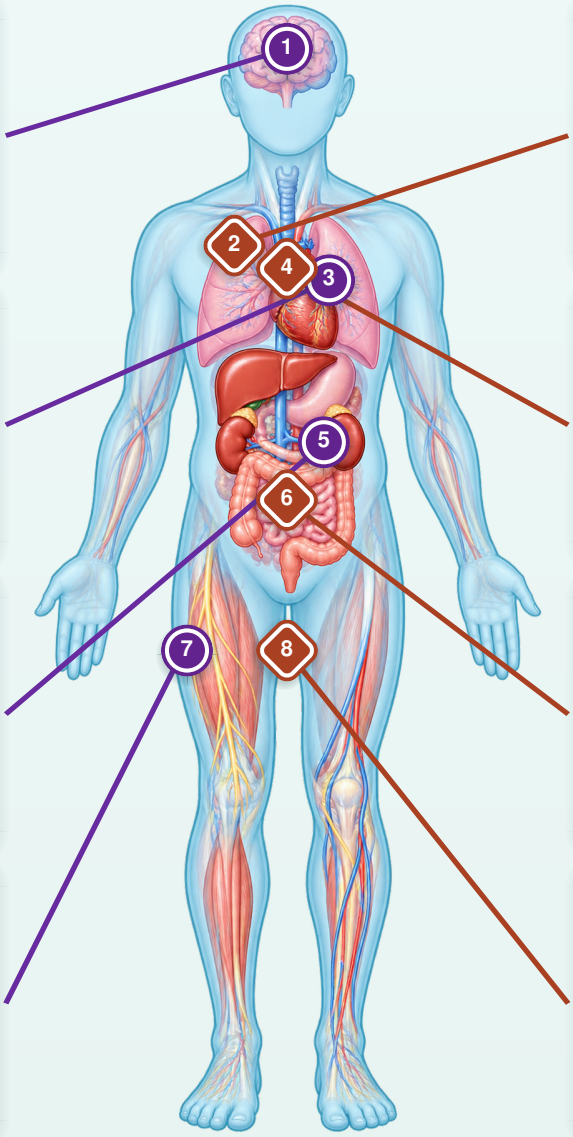
Bevacizumab

Protein in urine may cause no early symptoms. Keep urine and kidney checks; report swelling, reduced urine or a major change.

7 Skin / wound healing

Bevacizumab

Tell the team about operations, dental procedures and wounds. Report wounds that open, fail to heal or show infection.



2 Blood / bleeding

Bevacizumab / lomustine

Report unusual bruising or bleeding. Heavy bleeding, sudden chest pain or collapse needs emergency help. Counts can fall weeks after lomustine.

4 Heart / vessels

Bevacizumab

New chest pain, a painful swollen leg or sudden one-sided weakness needs urgent advice. Severe chest pain or collapse needs emergency help.

6 Abdomen / bowel

Bevacizumab / lomustine

Severe or persistent abdominal pain, vomiting or blood in stool needs urgent advice. Follow the prescribed nausea and bowel plan.

8 Fertility / pregnancy

Anticancer treatment

Discuss fertility and contraception before treatment. Do not breastfeed during therapy; obtain written medicine-specific advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Your whole treatment course

42-DAY CYCLE

Lomustine + bevacizumab 5 mg/kg

Up to 6 cycles | **42 days: lomustine once D1; bevacizumab IV D1, 15 + 29.**

Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1, 15 and 29. Up to six cycles are planned, with review before each treatment.

Each start, visit, delay or dose change is confirmed by your team. The card records treatment and does not authorise another dose.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Tests, symptoms and treatment decisions

Medicine-safety checks, tumour-tissue testing and response imaging answer different questions. Product-information additions are labelled. No universal blood tumour-marker panel or scan interval is implied.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00742 v3 · current Irish regimen · NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference · Lomustine medac current product information · January 2026 · EMA Avastin product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Lomustine + bevacizumab 5 mg/kg: treatment and checks

NCCP 00742: recurrent malignant glioblastoma; bevacizumab use is off-label.

Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1, 15 and 29. Up to six cycles are planned, with review before each treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Next treatment / response	After a pause or stop
Bloods / medicine safety NCCP 00742	Blood count, kidney/ liver tests, glucose, urine protein, blood pressure and cardiac history/examination; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment.	Regular blood count, kidney/liver tests and urine protein; blood pressure before each bevacizumab dose and after treatment. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment.	Keep additional tests even on days without an anticancer dose.	Team reviews current and previous results, symptoms and dose history.	Agree continuing checks; lomustine effects may persist.
Neurology / medicines / function NCCP 00742	Record usual symptoms, medicines, daily function and support needs.	Review headache, seizures, speech, strength, cognition and any steroid plan.	Report new changes early; do not wait for an MRI.	Discuss treatment tolerance, priorities and the next authorised dose.	Agree rehabilitation, symptom support and contacts.
Response / appointments NCCP 00742	Confirm exact diagnosis and baseline imaging.	Imaging and clinical review follow the specialist plan.	Record bookings, result-review contacts and questions.	Interpret scans with symptoms, treatment effects and medicines.	Agree the next imaging and clinical review.

What each check is for

1

Lomustine + bevacizumab 5 mg/kg

42-day cycle

Lomustine chemotherapy and bevacizumab targeted therapy are used together to help control recurrent glioblastoma. Discuss the expected benefit, symptom goals and treatment burden with your team.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00742

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.39

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1, 15 and 29. Up to six cycles are planned, with review before each treatment.

Treatment timing: displayed days 1–42

Treatment or check	Days in this window	What happens
Lomustine · one oral dose	Day 1	Take once on Day 1 only; use the newly authorised prescription.
Bevacizumab infusion	Day 1	Blood pressure, urine and safety review before treatment; observation during and after.
Bevacizumab infusion	Day 15	Blood pressure, urine and safety review before treatment; observation during and after.
Bevacizumab infusion	Day 29	Blood pressure, urine and safety review before treatment; observation during and after.
Continuing blood-count checks	Days 1–42	Lomustine SmPC advises weekly counts for at least six weeks; arrange all checks.
Symptoms and support between visits	Days 1–42	Keep the written emergency plan and do not wait for the next treatment to report changes.

A treatment record is not a prescription: The team may delay, alter or stop a component after reviewing symptoms, blood tests, organ function and previous toxicity. Do not restart a held medicine or add a replacement dose yourself.

Lomustine is one dose, not daily: Only the authorised Day 1 dose belongs to this cycle. Do not take a further course before six weeks and a new team decision. Taking repeated daily doses can cause serious harm. If a dose is missed or vomited, contact the team or pharmacist before taking any more. If an extra dose is taken, seek urgent clinical advice straight away even if you feel well; do not wait for symptoms.

Delayed marrow effects: Current lomustine product information advises blood counts weekly for at least six weeks after a dose. Low counts may be delayed and cumulative, so keep checks even when you feel well or the next treatment is postponed.

Infusion visits and procedures: Bevacizumab needs review around surgery and wound healing. Tell the team before an operation, dental procedure or other intervention, and do not assume an infusion is safe because a calendar interval has passed.

> **Why these bands stay broad:** Days without a scheduled dose still need symptom monitoring and planned tests. Bevacizumab infusion visits are not separate lomustine cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00742 v3 · current Irish regimen ↗

There are no guaranteed "safe days". Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions.

Complete timing and symptom notes

Day 1: Two medicines, different routes

Lomustine is a single oral dose; bevacizumab is an infusion. Keep the anti-sickness plan for lomustine. Confirm cycle number and the exact capsule supply before taking it.

Day 2–42: Separate infusion visits and continuing checks

Return for bevacizumab on Day 15 and Day 29. No extra lomustine is scheduled. Continue blood-pressure, urine and blood checks. Report symptoms and planned surgery; each medicine has its own safety decision.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking the treatment

Take lomustine only once on the authorised Day 1. The pharmacist must confirm the total dose, capsule strengths, how to take it and anti-sickness timing. Do not open or crush capsules or take another dose to replace a missed or vomited one without advice. Bevacizumab is given into a vein. The usual first infusion lasts 90 minutes; later infusions may take 60 and then 30 minutes if tolerated. Checks and observation add time. Report flushing, rash, breathing difficulty, dizziness or other new symptoms immediately.

STEP 2

Missed, vomited or held medicine

If a capsule dose is missed or vomited, call the oncology team or pharmacist before taking more. Do not double up or use leftover capsules. An infusion or capsule held for toxicity needs a new team decision.

STEP 3

Medicine and supplement review

Bring all prescription, over-the-counter and herbal medicines, vitamins and supplements to review. Include steroids, seizure medicines, anticoagulants and any recent antibiotics or antifungals. Tell the team before starting or stopping a medicine.

STEP 4

Surgery, procedures and wounds

Tell all treating teams that you receive bevacizumab. Major surgery needs an appropriate treatment interval and a fully healed wound before restarting; minor and dental procedures also need individual advice. Do not decide timing yourself or rely on the wallet stamp.

STEP 5

Capsule supply and handling

Lomustine is supplied as an unlicensed medicine in this NCCP protocol. Confirm pharmacy supply in advance. Keep the labelled pack away from children and do not open or crush capsules. Return unused chemotherapy to the pharmacy; contact staff if swallowing or handling is difficult.

STEP 6

Pregnancy, breastfeeding and fertility

Discuss fertility preservation and contraception before starting. Treatment can harm a pregnancy. Do not breastfeed during treatment. Ask for the written medicine-specific precautions and the required periods after stopping; these may differ between products.

STEP 7

Appointments and transport

Before starting: Blood count, kidney/liver tests, glucose, urine protein, blood pressure and cardiac history/examination; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. During treatment: Regular blood count, kidney/liver tests and urine protein; blood pressure before each bevacizumab dose and after treatment. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. MRI and neurological review follow your individual treatment plan. Echocardiography is considered only when indicated, including previous anthracycline treatment or chest-wall radiotherapy. If taking warfarin, INR is checked weekly until stable and before subsequent cycles.

STEP 8

Help with the daily plan

Agree who can help with transport, prescriptions, phone calls and recording symptoms if memory, vision, weakness or fatigue makes them difficult. Your consent and preferences guide what is shared. Keep the printed schedule and actual urgent contacts together.

STEP 9

Off-label treatment and infusion decisions

Bevacizumab for glioblastoma is outside its licensed indication in Ireland. The team should explain the reason for using it, expected benefit, uncertainty and alternatives as part of consent. Any rapid infusion outside the standard schedule has a separate local consent and governance process.

Sources: NCCP 00742 v3 · current Irish regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Lomustine nausea prevention

Lomustine has moderate-to-high oral emetic potential. Take the prescribed prevention and rescue plan around the single dose. Contact the team if nausea or vomiting prevents fluids or essential medicines; do not automatically repeat a vomited chemotherapy dose.

2 Bevacizumab support is separate

Bevacizumab alone has minimal emetic potential. Routine anti-sickness drugs or steroid premedication are not automatically required for every infusion; the team may prescribe specific medicines after a previous reaction or for another reason.

3 Blood pressure, urine and wound care

Keep prescribed blood-pressure medicine and urine checks. Follow the team's plan for any home measurements. Report wounds and upcoming surgery or dental treatment early, so timing and healing can be assessed before another infusion.

4 Separate steroid, seizure and bowel plans

Keep steroids and seizure medicines on their own written schedules if prescribed. Ask what to do after a missed or vomited dose. Other medicines can worsen constipation; agree a bowel plan and have all new medicines checked for interactions.

Sources: nccp · lomustine-smpc · avastin

TREATMENT REFERENCE · 01

Your treatment and its purpose

Keep the exact diagnosis, regimen and treatment goal together.

Why this treatment?

Lomustine chemotherapy and bevacizumab targeted therapy are used together to help control recurrent glioblastoma. Discuss the expected benefit, symptom goals and treatment burden with your team.

Sources: nccp

What each medicine does

Lomustine: Lomustine is an oral nitrosourea chemotherapy that can reach the brain. It damages DNA and interferes with cell division. Its effects on healthy blood-forming cells can appear several weeks after the dose and can build up over successive cycles. This explains the long gap between doses and the need for continuing blood tests. Bevacizumab: Bevacizumab binds VEGF, a signal involved in making and maintaining blood vessels. Blocking this signal can change the blood supply and leakiness of tumour vessels. It can also affect normal vessels, which helps explain high blood pressure, protein in the urine, bleeding, clots and slower wound healing. A scan change does not by itself prove that the tumour has stopped growing.

Sources: nccp · lomustine-smpc · avastin

Your schedule

Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1, 15 and 29. Up to six cycles are planned, with review before each treatment.

Sources: nccp

Evidence and uncertainty

Adding bevacizumab delayed measured progression in the trial but did not improve overall survival. Treatment goals and burdens need an individual discussion. The trial used bevacizumab 10 mg/kg every two weeks, not this NCCP 5 mg/kg every two weeks schedule. No trial survival or response number is assigned to this dose schedule. Trial lomustine dose escalation and treatment duration do not replace the current NCCP regimen. Historical pathology, patient selection and later treatments limit individual prediction.

Sources: eortc26101 · eortc-methods · nccp

Consent and decisions

Discuss the exact diagnosis, treatment goal, evidence limits, alternatives and burdens; medicine-specific risks, monitoring, fertility and pregnancy precautions; any unlicensed supply or off-label use; and practical support. Keep steroid and seizure plans separate from anticancer treatment.

Sources: nccp

TREATMENT REFERENCE · 02

Taking treatment and organising visits

Different medicines and support plans have different days.

Taking the treatment

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Sources: nccp · lomustine-smpc · avastin

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Sources: nccp · lomustine-smpc

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Sources: nccp · lomustine-smpc · avastin

Help with the daily plan

Agree who can help with transport, prescriptions, phone calls and recording symptoms if memory, vision, weakness or fatigue makes them difficult. Your consent and preferences guide what is shared. Keep the printed schedule and actual urgent contacts together.

Sources: nccn-patient

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Sources: nccp · avastin

TREATMENT REFERENCE · 03

Safety tests, support and urgent symptoms

A treatment-free day is still part of the care plan.

Which tests and when?

Before starting: Blood count, kidney/liver tests, glucose, urine protein, blood pressure and cardiac history/examination; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. During treatment: Regular blood count, kidney/liver tests and urine protein; blood pressure before each bevacizumab dose and after treatment. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. MRI and neurological review follow your individual treatment plan. Echocardiography is considered only when indicated, including previous anthracycline treatment or chest-wall radiotherapy. If taking warfarin, INR is checked weekly until stable and before subsequent cycles.

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Lomustine nausea prevention

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Sources: nccp · lomustine-smpc

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Sources: nccp · avastin

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Keep prescribed blood-pressure medicine and urine checks. Follow the team's plan for any home measurements. Report wounds and upcoming surgery or dental treatment early, so timing and healing can be assessed before another infusion.

Sources: nccp · avastin

Separate steroid, seizure and bowel plans

Keep steroids and seizure medicines on their own written schedules if prescribed. Ask what to do after a missed or vomited dose. Other medicines can worsen constipation; agree a bowel plan and have all new medicines checked for interactions.

Sources: nccp · nhs-dexamethasone · nccn-patient

When to call

Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Call urgently for bleeding, severe abdominal pain, chest symptoms, new headache with visual changes or wound problems. Tell the team about planned surgery before treatment.

Sources: nccp · lomustine-smpc · avastin · nci-cns

TREATMENT REFERENCE · 04

Understanding your particular glioma

Your diagnosis includes the tumour's cells, molecular findings and behaviour, not just the word brain cancer.

The name and location

Gliomas begin in supporting cells of the brain or spinal cord. Astrocytoma, oligodendroglioma and glioblastoma are different diagnoses. A tumour that spread to the brain from another organ is a brain metastasis and is not covered by these glioma regimens. Record the exact integrated diagnosis and where it is located.

Sources: nccn-patient

Grade is not the same as stage

Grade describes features linked to how a tumour behaves. Adult gliomas are usually discussed by their type, grade and molecular findings rather than the stage system used for many other cancers. Grade 4 does not mean cancer has spread through the body. Ask your specialist to explain your own report and any older terminology in the treatment protocol.

Sources: nccn-patient · nci-cns

IDH and 1p/19q

IDH1 or IDH2 changes and loss of parts of chromosomes 1p and 19q help classify particular gliomas. The combination of findings matters: one result should not be read in isolation. These tests are usually performed on tumour tissue. They do not automatically mean a change has been inherited or that relatives need testing.

Sources: nccn-patient

MGMT promoter methylation

MGMT is involved in repairing certain DNA damage. Methylation of its promoter can reduce that repair activity and helps the team discuss the likely benefit of temozolomide in relevant settings. It is not a blood tumour marker, a percentage of tumour left, or an individual survival prediction. Record the reported result and ask how it affects your treatment choice.

Sources: nccn-patient

A useful diagnosis record

Keep the biopsy or operation date, pathology wording, grade, IDH result, 1p/19q result and MGMT result if tested. Mark a result as pending or not applicable rather than assuming it is negative. Ask whether specialist pathology review or further testing would change the plan.

Sources: nccn-patient

TREATMENT REFERENCE · 05

Tests and scans: what each one tells you

Safety blood tests, tumour tissue tests and MRI answer different questions.

Safety blood tests

A full blood count checks infection-fighting white cells, oxygen-carrying red cells and platelets that help stop bleeding. Kidney and liver tests help assess treatment safety. Attend the regimen's required tests even when you feel well; a reassuring scan does not replace them. The schedule in this guide separates checks before, during and after treatment.

Sources: nccp · nccn-patient

MRI and neurological review

MRI is the main scan used to assess many gliomas. Your team also considers symptoms, strength, movement, speech, memory, daily function and medicines such as steroids. Tell staff before MRI about implants or metal, kidney problems, previous contrast reactions or difficulty lying still. Ask about support if scans make you anxious.

Sources: nccn-patient

Changes on a scan need interpretation

After treatment, inflammation and other treatment effects can resemble tumour growth on MRI. This is sometimes called pseudoprogression. The team may need comparison scans, specialist imaging or further assessment before deciding what a change means. New or worsening symptoms still need prompt review; do not assume they are harmless treatment effects.

Sources: nccn-patient

An individual follow-up timetable

Write the next MRI date, what question it should answer and who will explain the result. Timing depends on your diagnosis, treatment phase and previous findings. General follow-up intervals in an international booklet are not automatically your schedule. These regimens do not require a generic CT chest/abdomen/pelvis or a blood tumour marker before every cycle.

Sources: nccn-patient

Keep results with decisions

Use the passport to distinguish a test being booked, completed and reviewed. Record the agreed next step and whom to call if an appointment or result is missing. Do not alter chemotherapy or steroids yourself in response to a number or scan report.

Sources: nccn-patient

TREATMENT REFERENCE · 06

Brain symptoms, medicines and when to call

Symptoms may reflect the tumour, swelling, treatment, another medicine or a separate illness.

Know your usual symptoms

Write down your usual headache, strength, walking, speech, vision, memory, mood, sleep and seizure pattern before treatment. Note new changes, when they started and how they affect daily tasks. With your permission, a trusted person can help spot changes or record what happened.

Sources: nccn-patient

Urgent neurological changes

Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions.

Sources: nccp · nccn-patient

Steroids need a separate written plan

Steroids may be prescribed to reduce swelling around the tumour. They are not automatically needed for everyone. Ask for the exact dose, time, review date and any taper plan. Do not stop or change them suddenly without advice. Report sleep or mood changes, increased thirst, infection symptoms or muscle weakness and ask which additional checks or protection you need.

Sources: nccn-patient · nhs-dexamethasone

Seizure medicines and rescue instructions

If prescribed, take seizure medicines on the agreed schedule. Ask what to do after a missed or vomited dose and whom to contact for breakthrough symptoms. Some medicines can affect mood, alertness or interact with treatment. Do not start, stop or switch them yourself. A rescue medicine needs an individual plan and training for whoever may give it.

Sources: nccn-patient

Safety at home and outside

Ask your clinical team about driving, work, bathing, swimming, heights and machinery in light of your symptoms and any seizures. Local medical and licensing rules need individual advice; this guide does not give a universal driving clearance date. Occupational therapy can help adapt daily routines and the home.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Your overall treatment plan and choices

Treatment should account for the tumour and the life you want to lead.

Understand the sequence

Surgery or biopsy, radiotherapy, chemotherapy and follow-up have different jobs. Ask your team to draw the intended sequence and identify where you are now. Concurrent treatment means treatments overlap; adjuvant treatment follows the main treatment. A change of phase may change the dose, blood tests and support medicines.

Sources: nccn-patient

Agree what treatment is trying to achieve

Ask whether the aim is to control the tumour, delay growth, relieve symptoms, preserve function or a combination of these. Discuss likely benefits, burdens, alternatives and what would prompt a pause or stop. The answer depends on the exact diagnosis, previous treatment and your priorities.

Sources: nccn-patient

Put evidence in context

Clinical trials describe groups of people and may use older pathology names or different schedules. They cannot predict one person’s response or lifespan. Ask how closely the evidence matches your tumour and treatment. No benefit number from one schedule should automatically be applied to another.

Sources: nccn-patient

Discuss options available to you

Ask about appropriate clinical trials, specialist opinions, symptom support and other treatment options. A medicine or device described in an international guide is not automatically licensed, funded or suitable in Ireland. The treating multidisciplinary team explains what is available and relevant.

Sources: nccn-patient

Make room for your priorities

Bring up fatigue, thinking, independence, family time, work, travel and side effects you most want to avoid. Ask for information at a pace and in a format that suits you. You may ask for a trusted person to join the discussion, written notes and time to reconsider decisions.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Living well, rehabilitation and support

Practical and symptom support can begin alongside anticancer treatment.

Thinking, communication and daily tasks

Tell the team about word-finding, attention, memory, reading, vision or planning difficulties. Rehabilitation may include physiotherapy, occupational therapy, speech and language therapy, or neuropsychological support. Written routines, rest breaks and help with medicine organisation can make day-to-day care more manageable.

Sources: nccn-patient

Food, nausea, bowels and energy

Report poor appetite, weight change, vomiting or constipation early. Ask for dietitian help if eating is difficult. Use the prescribed anti-sickness and bowel plan; do not wait until symptoms are severe. Choose activity that is safe for your balance and strength and balance it with rest. Ask about treatable causes of fatigue.

Sources: nccp · nccn-patient

Emotional support and carers

Anxiety, low mood, changes in behaviour and uncertainty can affect both patients and carers. Tell the team what has changed and ask for appropriate psychological or social support. Carers may need their own information, practical help and breaks. Agree who the team may share information with.

Sources: nccn-patient

Work, transport and finances

Ask the specialist nurse or social worker about getting to treatment, time off work, benefits, caring responsibilities and support at home. Plan who can help with medicines, symptom recording and appointments if your memory, vision or fatigue makes these difficult.

Sources: nccn-patient

Supportive care and future wishes

Supportive and palliative care can help with symptoms and quality of life while active treatment continues. You can discuss future care preferences and who you want involved in decisions at a time that suits you. Ask the team for Irish advice and documentation rather than using legal or hospice rules from a US booklet.

Sources: nccn-patient

Information that works on paper

Keep the printed schedule, current medicine list, tests, contact numbers and emergency instructions together. This handbook can be read in sequence without internet access. The wallet card and optional QR code help others find the regimen information; they do not replace your prescription or emergency plan.

Sources: nccn-patient

EORTC 26101: evidence and schedule limits

The randomised trial compared lomustine plus bevacizumab with lomustine alone after glioblastoma progression.

Adding bevacizumab delayed measured progression in the trial but did not improve overall survival. Treatment goals and burdens need an individual discussion.

Sources: eortc26101 · eortc-methods · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

What is my exact diagnosis and how does this regimen fit?

NCCP 00742: recurrent malignant glioblastoma; bevacizumab use is off-label. Ask for the pathology, relevant molecular findings and the treatment goal in plain language.

Which medicines do I receive on which days?

Each 42-day cycle has lomustine capsules taken once on Day 1 and bevacizumab infusions on Days 1, 15 and 29. Up to six cycles are planned, with review before each treatment.

Which tests must happen before the next treatment?

Before starting: Blood count, kidney/liver tests, glucose, urine protein, blood pressure and cardiac history/examination; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. During treatment: Regular blood count, kidney/liver tests and urine protein; blood pressure before each bevacizumab dose and after treatment. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. MRI and neurological review follow your individual treatment plan. Echocardiography is considered only when indicated, including previous anthracycline treatment or chest-wall radiotherapy. If taking warfarin, INR is checked weekly until stable and before subsequent cycles.

How do I handle a missed, vomited or held dose?

Contact the oncology team or pharmacist for medicine-specific instructions. Do not double a dose or replace treatment that was held for toxicity.

Who helps with steroids, seizures, hearing, memory or daily tasks?

Ask for written medicine plans and a named contact. Rehabilitation, practical support and a trusted helper may support the daily plan.

Which symptoms mean I should call now?

Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Call urgently for bleeding, severe abdominal pain, chest symptoms, new headache with visual changes or wound problems. Tell the team about planned surgery before treatment.

When do we review whether treatment is helping?

Agree the next clinical and imaging review, what would count as benefit, how side effects will be weighed and what would lead to continuing, changing or stopping.

> **Try explaining the plan back:** Can you show the day for each medicine, your next safety tests, the current support plan and whom to call for fever, new neurological symptoms or another urgent change?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00742 v3. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00742 v3 · current Irish regimen
- NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference
- NCI: adult central nervous system tumours · diagnosis and grade education
- NHS: dexamethasone · use and stopping safely
- Lomustine medac current product information · January 2026
- EMA Avastin product information and patient leaflet
- Wick 2017: lomustine and bevacizumab in progressive glioblastoma
- EORTC 26101 original methods · trial doses and schedules

Written source addresses

https://healthservice.hse.ie/documents/6874/742_V3_Lomustine_and_Bevacizumab_5.pdf

https://www.nccn.org/patients/guidelines/content/PDF/brain-gliomas-patient.pdf

https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/common_SPCs/Lomustine_medac-spc-common.pdf

https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf

https://www.cancer.gov/types/brain/patient/adult-brain-treatment-pdq

https://www.nhs.uk/medicines/dexamethasone-tablets-and-liquid/how-and-when-to-take-dexamethasone-tablets-and-liquid/

https://pubmed.ncbi.nlm.nih.gov/29141164/

https://www.nejm.org/doi/full/10.1056/NEJMoa1707358